

SENSITISATION WORKSHOP TO KICK-START THE IMPLEMENTATION OF THE PHARMACIST
CONSULTANT CADRE WITHIN THE HEALTHCARE SYSTEM IN NIGERIA

EFFECTIVE IMPLEMENTATION OF CONSULTANT PHARMACY CADRE FOR IMPROVED HEALTHCARE DELIVERY IN NIGERIA

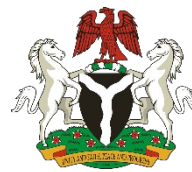
Prof. Ahmed Tijjani Mora

B.Sc (Pharm); MBA; FPCPharm; PhD; FPSN; FICEN; FIHMN; FNIM; mni

(Kayayen Sarkin Musulmi),

The Chairman of the Governing Council of the Pharmacists Council of
Nigeria (PCN)

Tuesday, 18th May 2021





PREAMBLE: Pharmacy profession

Pharmacy is a healthcare discipline and a profession.

It is a profession based on the following criteria:

Made up of a specialized body of knowledge

Having a registration body established by Law (i.e., PCN)

License for the practitioners to practice the profession

Oath-taking among the practitioners

Code of Conduct

Body of Ethics in the practice of the profession

Altruism in practice



PREAMBLE: Pharmacy profession

As a healthcare discipline and profession, the pharmacist is expected to:

Practice the profession based on the provisions of the pharmacy oath in Nigeria.

Practice the profession based on the profession's codes of conduct.

Practice the profession along with other members of the healthcare team in a multidisciplinary approach in any of the approved healthcare provider facilities whether public or private.



PREAMBLE: Healthcare regulatory agencies

Healthcare professions are regulated and controlled by the following Agencies: These bodies defined the various instructions to be necessarily acquired by persons wishing to practice the profession either as Dispensers, Chemists and Druggists, Diploma in Pharmacy, to BSc. (Pharmacy), BPharm, the PharmD degrees, etc.,

Medical and Dental Council of Nigeria (MDCN)

Nursing and Midwifery Council of Nigeria (NMCN)

Medical Laboratory Scientists Council of Nigeria (MLSCN)

Radiographers Registration Board of Nigeria (RRBN)

Community Health Practitioners Registration Board of Nigeria
(CHPRN)



PREAMBLE: Healthcare regulatory agencies

Optometrists and Dispensing Opticians Registration Board of Nigeria
(ODORBN)

Medical Rehabilitation Therapist Board of Nigeria (MRTBN)

Dental Therapists Registration Board of Nigeria (DTRBN)

Dental Technologists Registration Board of Nigeria (DTRBN)

Health Record Officers Registration Board of Nigeria (HRORBN)

Institute of Chartered Chemists of Nigeria (ICCON)

Institute of Public Analysts of Nigeria (IPAN)

Environmental Health Officers Registration Board of Nigeria
(EHORBN)



PREAMBLE: Functions of Healthcare regulatory agencies

All these healthcare regulatory agencies:

Are supervised by the Federal Ministry of Health. Have Governing Councils under the Chairmanship of a Chairman appointed by the President of Nigeria to supervise the activities of the Registries. are run by the Registrars.

Regulate and control the education, trainings and practices of their various professionals

Overall, the purpose is to have a unified, coordinated and integrated healthcare services across health facilities in Nigeria.



PREAMBLE:

Peculiarities of the pharmacy profession

There are certain peculiarities of the pharmacy profession and its practice that are different from the other healthcare professions.

These are:

All pharmaceutical products must be registered with NAFDAC. All premises where pharmaceutical products are manufactured, offered for sale or dispensed must be inspected, approved, registered and licensed by the Pharmacists Council of Nigeria (PCN). All Pharmaceutical Premises must be registered by a pharmacist who is registered and licensed by the PCN. Ethical pharmaceutical products must only be dispensed by the pharmacist on the presentation of a prescription by a medical practitioner or a dentist.



PREAMBLE:

Peculiarities of the pharmacy profession

Narcotics and products containing psychotropic substances are dispensed using Poison Disposal Register.

Pharmaceutical products have expiry dates after which they cannot be dispensed or offered for sale.

Pharmaceutical products listed on the prescriptions cannot be dispensed in quantities in excess of what the prescriber indicated.

Only prescriptions generated from within healthcare provider facility can be filled in the pharmaceutical services department of the facility.



PREAMBLE: History of pharmacy regulation in Nigeria

The regulation and control of pharmacy education, training and practice in Nigeria has gone through different Agencies established thorough Ordinances, Laws, Decrees and LFN as in the: These bodies defined the various instructions to be necessarily acquired by persons wishing to practice the profession either as Dispensers, Chemists and Druggists, Diploma in Pharmacy, to BSc. (Pharmacy), BPharm, the PharmD degrees,



- Board of Medical Examiners
- Pharmacy Board of Nigeria
- Pharmacists Board of Nigeria
- Pharmacists Council of Nigeria



PREAMBLE: Establishment of the WAPCP

The establishment of the West African Pharmaceutical Federation in 1976 in Monrovia, Liberia and which in 1991 became the West African Postgraduate College of Pharmacists (WAPCP), that for the first time:

Provided a unified body of curricula for the registered pharmacists within the five Anglophone West African countries (Ghana, Liberia, Nigeria, Sierra Leone and The Gambia) towards the acquisition of the Fellowship of the College.

The curricula of the Fellowship program has advanced the quality of pharmaceutical services for its pharmacy graduates, who recently acquired a consultant status as approved by the Head of the Civil Service of the Federation following recommendations by the National Council on Establishment.

It is within the context of the aforementioned advancement of learning, teaching and research and practices in respect of the College graduates that led to the Pharmacy consultant cadre in Nigeria that this Paper would be presented.



THE WEST AFRICAN POSTGRADUATE COLLEGE OF PHARMACISTS

The aims and objectives of WAPCP.

To advance:

knowledge and skills of practicing pharmacists through postgraduate education and training.

advance the knowledge of the application of pharmacy in the health care system.

To promote:

research in pharmacy and to advance pharmacy practice in specialized disciplines.

Ethical Pharmacy Practice in the region.

pharmaceutical care practice.

the integration of traditional and herbal medicines into the National Health Systems.



THE WEST AFRICAN POSTGRADUATE COLLEGE OF PHARMACISTS

The aims and objectives of WAPCP.

To promote:

Public Health Pharmacy.

harmonization of pharmacy education in the region.

harmonization of pharmaceutical legislation and development of National Drug Policies in the region.

Health Commodities and Drug Supply Chain Management.

local Drug Production and Quality Assurance of Health Commodities, Drugs and Services.

pharmaceutical care practice.

the integration of traditional and herbal medicines into the National Health Systems.



THE WEST AFRICAN POSTGRADUATE COLLEGE OF PHARMACISTS

The aims and objectives of WAPCP.

To identify and accredit institutions for professional postgraduate education and residency training.

To conduct examinations and award Postgraduate Fellowship, Diploma and Certificate.

To identify and accredit institutions for professional postgraduate education and residency training.

To co-operate with other health professionals, scientists and organizations in activities that will ensure maintenance of high standard of pharmacy practice and health care delivery.

To collaborate with Sister Colleges and National Colleges.

To engage in other activities that will ensure the furtherance of the aims and objectives enumerated above.



THE WEST AFRICAN POSTGRADUATE COLLEGE OF PHARMACISTS

The five faculties of the WAPCP.

Faculty of Drugs Production and Quality Assurance.

Faculty of Social and Administrative Pharmacy.

Faculty of Clinical Pharmacy.

Faculty of Community Pharmacy.

Faculty of Public Health Pharmacy.

These Faculties provide instructions to students enrolled for the Fellowship programs leading towards their graduation and the issuance of certificates designated as Fellows of the Postgraduate College of Pharmacists (FPCPharm).



THE WEST AFRICAN POSTGRADUATE COLLEGE OF PHARMACISTS

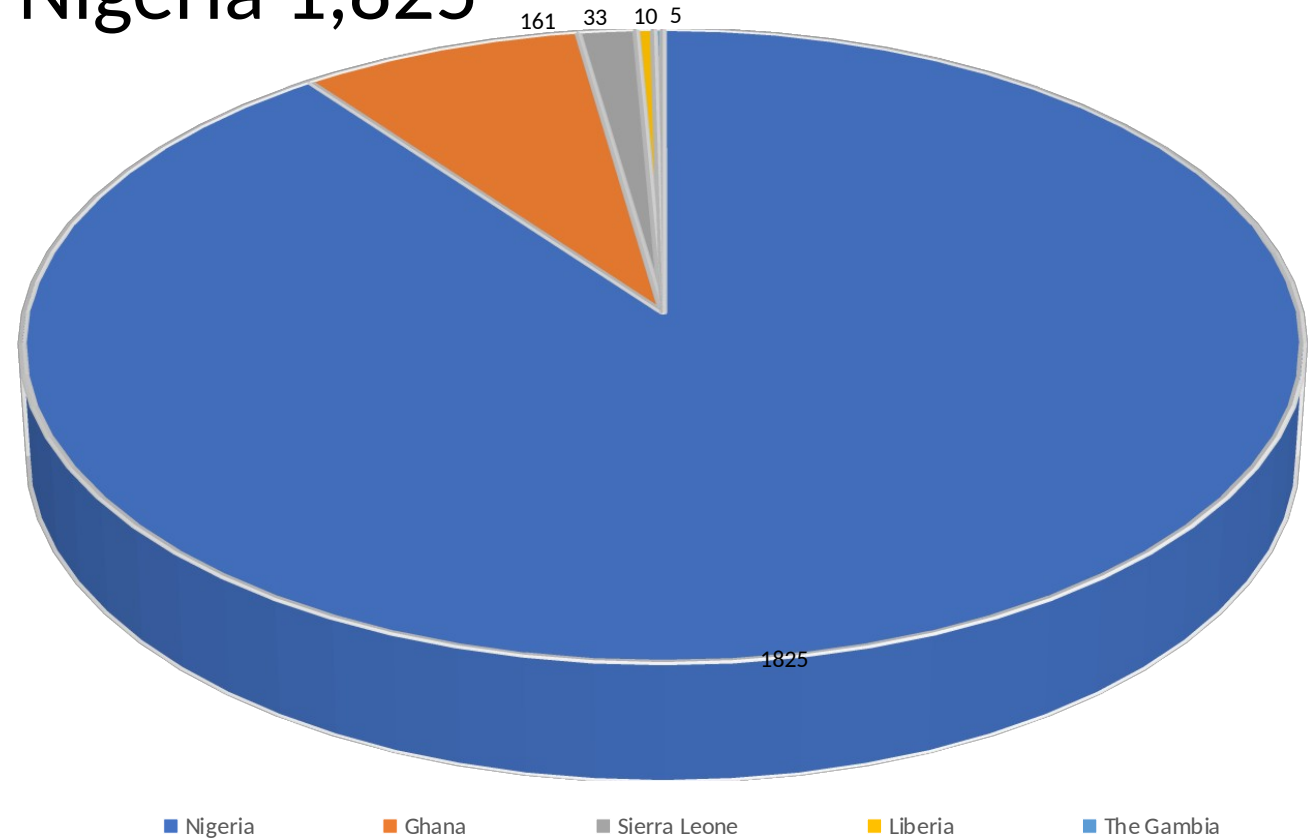
Number of Fellows to date Nigeria 1,825

Ghana 161

Sierra Leone 33

Liberia 10

The Gambia 5



Source: WAPCP (2021)



THE ROLE OF THE PCN IN THE REGULATION OF THE WAPCP

Background.

In September 2006, the Registry of the PCN received the “Report of the Pharmacists Council of Nigeria (PCN) visitation Panel to West African Post-graduate College of Pharmacists (WAPCP)”.

The Council had earlier received a letter from the West African Post-graduate College of Pharmacists requesting for recognition of the Fellowship program of the College.

A Panel under Prof. E. N. Sokomba as Chairman was inaugurated along with three others including the Secretary with the following mandate: To examine;

the Curriculum of the Fellowship Programme of the WAPCP human and material resources available for the execution of the programme

the mode of examinations, marking schemes, project reports, reports of preceptors, examiners reports, and any other relevant documents related to examinations To make Recommendations that may assist the PCN on the request for recognition of the Fellowship.



THE ROLE OF THE PCN IN THE REGULATION OF THE WAPCP

Accreditation of the WAPCP.

In concluding their assignment, the Committee wrote in their report signed on 7th September 2006 and stated in part as follow: "The College is providing Pharmacists in the sub-region the opportunity for postgraduate professional training through the Fellowship programme and this effort should be sustained and encouraged. There is a need for the programme to be recognized by the PCN to facilitate appropriate placement of the Fellows in the Scheme of Service along with commensurate remuneration. We therefore recommend recognition of the Fellowship programme while the College should be advised to remedy the deficiencies highlighted by the Panel". The Council accepted the report and the prayer of the Panel, and the College was accredited.



APPROVED SCHEME OF SERVICE FOR PHARMACIST CONSULTANT CADRE IN THE FEDERAL/PUBLIC SERVICE

On 11th September 2020, the Head of Civil Service of the Federation, Dr. Folasade Yemi-Esan in a signed circular (HCSF/CSO/HRM/1274/T3) conveyed the approval of the pharmacist consultant cadre in the Federal Civil/Public Service by the National Council on Establishment (NCE) at its 41st meeting held from Monday, 4th to Friday, 8th November 2019, in Asaba, Delta State.

The NCE approved the Pharmacist Consultant cadre for inclusion in the Schemes of Service. The cadre is structured on SGL15-17 as follow:

Consultant Pharmacist SGL15

Consultant Pharmacist (Special GD II) SGL16

Consultant Pharmacist (Special GD I) SGL17

The PCN immediately wrote a lengthy letter of appreciation to the Head of Service for making a 30-year struggle by the PCN and other stakeholders of pharmacy for the consultant cadre a reality.



Induction of the Consultant Pharmacists in Nigeria

A consultant is defined as: "an expert qualified and experienced in a specific discipline or field to give advice or service to a person, organization or government..." (WHPA 2007)

A consultant is equipped with the paraphernalia that would assist him to excel in these identified areas.

The PCN Act, LFN, No. 17 of 2004 empowers the PCN to regulate and control pharmacy education and training as well as practice in Nigeria in all aspects and ramifications.

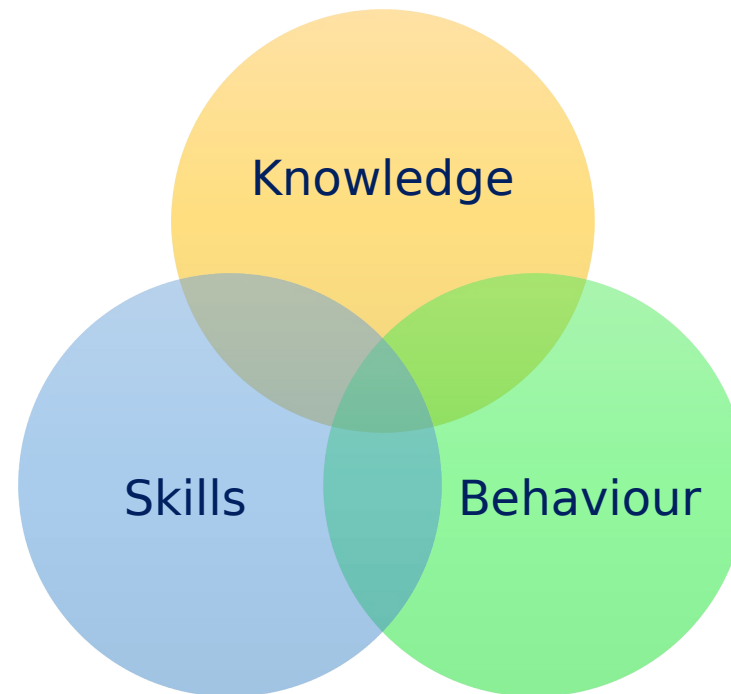
Therefore, it has become the responsibility of the PCN as the regulator of Pharmacy education and training as well as practice to ensure proper induction of the Consultant Pharmacists in updated education, skills and behavior.



The Consultant Pharmacist in the Dispensation of Knowledge, Skills, behaviors in Healthcare Facilities

The Consultant must answer true to his/her calling as someone who would be consulted to give superior advice and/or judgement on aspects of Pharmacy appropriate to his/her specialty.

The three attributes: knowledge, skills and behaviors must be always exhibited by the Consultant Pharmacist in the specialized area of Pharmacy practice he/she finds himself.





The Consultant Pharmacist in the Dispensation of Knowledge, Skills, behaviors in Healthcare Facilities

Knowledge

A Consultant must always be knowledgeable and ready to offer professional inputs whenever required.

Knowledge must be updated periodically through participation in the Mandatory Continuing Professional Development programme (MCPD) and continuous self-improvement.

The knowledge must be seen to be made available to/or in the following:
Fellow colleagues in the practice area (hospital, community, manufacturing or academic).

Fellow healthcare professionals in reviewing patient's medication or in strengthening the system.



The Consultant Pharmacist in the Dispensation of Knowledge, Skills, behaviors in Healthcare Facilities

Knowledge

Mentoring of younger Pharmacists especially in therapy, during interactions, contraindications, dosing, side effects and observation of clinical outcomes in patients.

Design of clinical trials, research studies in specific areas to have evidence-based data for decision-making.

Presentations in hospital settings in respect of new pharmaceutical products, preparation of Hospital Drug Formularies, Drug Bulletin review, and classification of pharmaceutical products.

Instructions as to the use of computer systems in dispensing of drugs to patients, referral information and dissemination of same to fellow colleagues and other healthcare professionals.



The Consultant Pharmacist in the Dispensation of Knowledge, Skills, behaviors in Healthcare Facilities

Skills

The ability to work as part of a team

Interpersonal and communication skills

Creativity and innovation

Problem-solving and strategic-planning ability

Analytical skills

Flexibility

Skills



The Consultant Pharmacist in the Dispensation of Knowledge, Skills, ~~behaviors in Healthcare Facilities~~

Behaviours:

The behavior of a Consultant Pharmacist must be distinct in any healthcare setting.

The saying that “first impression” matters is germane here. A Consultant Pharmacist must present himself in a manner to have the confidence of his/her co-healthcare professionals, the patients and members of the public.

He should be seen as an exemplary good behavior by co-healthcare professionals, the patients and members of the public.

Behaviours:

- Dressing automatically takes a leading position in terms of behavior expected of a Consultant Pharmacist.





The Consultant Pharmacist in the Dispensation of Knowledge, Skills, behaviors in Healthcare Facilities

Other attributes required are as follows:

Listen first if someone is asking for an advice, he or she has problems to be solved. The Consultant knowing his or her value.

It is okay to say “No” if a Consultant does not agree with something.

Offer creative solutions. There must be originality and creativity in how a Consultant solves a problem.

The Consultant Pharmacist must always exude confidence.



Integrating Knowledge, Skills, behaviors in Ethical Practice by a consultant

A good example of Integrating Knowledge, Skills and behavior in Ethical Practice by consultant is in the table below:

Knowledge	Skills	Behaviours
Is knowledgeable about human rights declarations and instruments (e.g. United Nations Universal Declaration of Human Rights and relevant professional declarations and statements). <i>Refer to the annex for all list of professional declarations and statements by WHPA member organizations.</i>	- Values and takes appropriate action to safeguard human rights. - Able to apply and adapt ethical principles and codes of practices to different work settings and clients needs.	- Upholds basic human rights and dignity at all times and reports observed violations as appropriate. - Conducts oneself with honesty and integrity and in accordance with the published Code of Ethics of one's profession where such exists.

Source: A core competency Framework for International Health Consultancy (WHPA, 2007)



Integrating Knowledge, Skills, behaviors in Ethical Practice by a consultant

Knowledge	Skills	Behaviours
- Is knowledge about professional codes of ethics, if applicable. - Is knowledgeable about information needs of clients and partners and about the legal ethical rules for disclosures. - Is knowledgeable about potentials conflicts of interest. - Is knowledgeable about ethical research conduct.		- Deals with others in an open and honest manner. - Is sensitive to the client's culture, traditions and patterns of decision-making. - Present own professional qualifications and credentials accurately. - Observes individual country requirements for conduct of research and obtains required clearance.

Source: A core competency Framework for International Health Consultancy (WHPA, 2007)



Conclusion, Recommendations and Implementation Strategies

Conclusion

Kudos must be given to the National Council on Establishment whose recommendation established the approved scheme of service for Pharmacist Consultant Cadre in the Federal Civil/Public Service.

With this development therefore, Pharmacists in the Civil /Public Service have joined their colleagues, the Medical Doctors in attaining Consultant Status all in the interest of the system and the patients.

Healthcare system in the world generally is dynamic and the challenges faced by mankind in terms of new diseases like the n COVID-19 as well as resurgence of others such as Tuberculosis, must be met with programmes and/or recognition of certain knowledge, skills, and behavior such as the approved Consultant cadre for the Pharmacists.

Nigeria has set the pace and it is only a matter of time before our colleagues in other Anglophone countries such as Ghana, Liberia, Sierra-lone and the Gambia as well as the Francophone too follow in the same way.



Conclusion, Recommendations and ~~Implementation Strategies~~

The focus should be towards producing International Health Consultants within the Pharmacy profession in the ECOWAS sub-region.

These are: “experts working in health services who have specialized knowledge (Pharmacy) experience and personal attributes that qualify them to provide advice or services (other than direct care) beyond home country (WHPA, 2007).”



Conclusion, Recommendations and Implementation Strategies

Recommendations and Implementation Strategies

1

Ministries, Departments, and Agencies (MDAs) at the Federal, States and Local Government levels should ensure that all Fellows of the WAPCP that have achieved the Consultant status are identified, recognized and appropriately remunerated.



Conclusion, Recommendations and Implementation Strategies

Implementation Strategies for recommendation No.1:

All Consultants Pharmacists to have their Licenses to Practice issued by the PCN to indicate their status as Consultants.

All Consultants to be remunerated as per their status in line with the approved provision of the Schemes of Service at the Federal, States and Local Governments as the case may be;

Office facilities of the Consultant Pharmacists within the healthcare provider facilities at the Federal, State and LGCs to be upgraded, remodeled or built and equipped to enable the Consultant Pharmacist offer the appropriate healthcare services.



Conclusion, Recommendations and ~~Implementation Strategies~~ Recommendations

2

The WACPC should draft an updated topics to be offered under a special Continuing Professional Development (CPD) Programme that must be accessed as capacity building programme by Consultants.



Conclusion, Recommendations and Implementation Strategies

Implementation Strategies for recommendation No. 2:

The WAPCP Secretariat to draft topics in the different Faculties and convey to the PCN which would in turn organize the CPD for the Consultants.

The Federal, States and Local Government healthcare provider facilities to ensure free access at reasonable times for the Consultants in the public health institutions that are staff of the Pharmaceutical Services Department for the benefits of the inpatients admitted in the wards of the facilities. Such easy access to be monitored by the PCN State Offices.

Consultant Pharmacists to periodically prepare Drug Bulletins within the healthcare facilities for information and easy reference by the Medical Practitioners, Dentists and other relevant healthcare professionals in prescribing, administering the medication or the appropriate use of medical devices.



Conclusion, Recommendations and Implementation Strategies

Recommendations

3

The organized private sector offering services such as pharmaceutical manufacturing, Drug Importation, Wholesaling Companies and Hospital Pharmacies should make adequate provision of recognition and remunerations for the Consultant Pharmacists in their employment.



Conclusion, Recommendations and Implementation Strategies

Implementation Strategies for recommendation No. 3:

The salary, welfare packages and other fringe benefits of the Consultant Pharmacist to be reviewed to reflect the status of the Consultant Pharmacist.

Office accommodation to be upgraded and equipped befitting the status of Consultant Pharmacist.

The organization to pay for the CPD for the Consultant pharmacist as and when due.



Conclusion, Recommendations and ~~Implementation Strategies~~

Recommendations

4

The Consultant Pharmacist should be readily identified in his/her place of work by way of dressing in clean white overall and with name tag with inscription.

Pharm. (Name), FPCPHarm
CONSULTANT PHARMACIST



Conclusion, Recommendations and ~~Implementation Strategies~~

Implementation Strategies for recommendation No. 4:

The WAPCP to produce customized overall of different sizes with excellent white cotton materials that must be procured and always worn by Consultant Pharmacists while at work.

Well laundered shirts, ties and polished covered shoes with socks to match to be always worn by the Consultant Pharmacist with their name tags while at work.

Multiple overall to be procured by every Consultant Pharmacist from the WAPCP to endure that they are washed periodically, and the other set(s) worn. The WAPCP to produce the name tags for distribution to the Consultant Pharmacists and which must be pinned on the front pocket.



Conclusion, Recommendations and Implementation Strategies

Recommendations

5

The MDAs and organized private sector offering healthcare services at different tiers of Government should make annual budgetary allocations for the Fellowship Training of Pharmacists in their employment once they had spent certain years of service in the MDAs.



Conclusion, Recommendations and Implementation Strategies

Implementation Strategies for recommendation No. 5:

Annual budgets to make provisions for Fellowship training of the Pharmacists that are employed in the healthcare provider facilities.

The Consultants Pharmacist to distinguish themselves in their carriage, dedication to duty and updated professional skills, expertise and competencies.

Consultant Pharmacists to have a platform of collaboration and harmonizing of competences at social media level, during the Annual scientific Conferences and at other recognized fora.



Conclusion, Recommendations and ~~Implementation Strategies~~

Implementation Strategies for recommendation No. 5:

Annual budgets to make provisions for Fellowship training of the Pharmacists that are employed in the healthcare provider facilities.

The Consultants Pharmacist to distinguish themselves in their carriage, dedication to duty and updated professional skills, expertise and competencies.

Consultant Pharmacists to have a platform of collaboration and harmonizing of competences at social media level, during the Annual scientific Conferences and at other recognized fora.



Conclusion, Recommendations and Implementation Strategies

I believe it is pertinent to end this presentation with a quote from Prof. J. E. Essien as follows:

“...In Nigeria today, there is a great need for enhancement of the status, knowledge and skills in Clinical Pharmacy through the biopharmaceutical sciences which form the basis of therapeutics in patient care services.

In the period 2006 to 2009, all the Universities and Institutions running courses or programmes in Medicine, Pharmacy and Allied disciplines came together in the United Kingdom. They organized heavy work programmes of conferences in Clinical Pharmacy and distributed information materials on the conferences throughout the United Kingdom and abroad. They offered Certificates, Diplomas, Degrees, etc. By 2009, they had persons who had qualified to be given Certificates, Degrees and Diplomas. In addition, there were individuals who received special award like “Independent Prescribers of Medicines”. Thus, the importance of Clinical Pharmacy was emphasized as desired knowledge in the practice of therapeutics in the Medical Schools, Pharmacies, Hospitals and Clinics...” Prof. E. E. Essien

*Thank
you*



Prof. Ahmed T. Mora, PhD, FPCPharm, mni
Tuesday, May 18th, 2021, Abuja, FCT.





REFERENCES

WHPA (2007); “A Core Competency Framework for International Health Consultants” a publication from the World Health Alliance (WHPA), Geneva, Switzerland.

PCN (2006); “Report of the Pharmacists Council of Nigeria (PCN) Visitation Panel to West African Postgraduate College of Pharmacists (WAPCP) (Unpublished).

CRHP (2008); Compilation of Acts Establishing Health Professional Regulatory Agencies in Nigeria (Unpublished).